

The Three Kinds of No

Your GLP-1 was denied. The next step depends entirely on which no you got — and only one of the three is final.

FROM A PHYSICIAN ASSISTANT WHO'S BEEN ON A GLP-1 HERSELF

A denial letter feels like a verdict. It usually isn't. Most people never find out **which kind** of no they were given — and the fix for one kind does nothing at all for another. Read the letter again and find the phrase.

NO #1 — “STEP THERAPY” (the fail-first no)

Your plan wants you to try — and not do well on — a different, usually cheaper option before it will cover the one your prescriber chose. It is not a judgment about whether you need treatment.

What to do

Bring the letter to your prescriber. If you have already tried the required alternative, or there is a medical reason you shouldn't, their office can file a **step-therapy exception** with your chart notes attached. This is a documented, routine process — prescriber offices do it every week.

NO #2 — “PRIOR AUTHORIZATION” (the paperwork no)

This is the most common one, and the most fixable: the plan says it did not receive enough documentation to approve coverage. Missing chart notes, a BMI that was never written down, a diagnosis code that didn't transmit. Annoying — and genuinely worth pursuing, because appeals exist precisely for this.

What to do

Call the member number on the letter and ask exactly what was missing — they are required to tell you the reason for the denial. Then ask your prescriber's office to resubmit with that specific gap filled. If it is denied again, you can file an **internal appeal** (your letter states the deadline — often 180 days), and if that fails, most plans must offer an **external review** by an independent reviewer who does not work for your insurer. People give up long before this step. The ones who don't are the ones who get the overturns.

NO #3 — “PLAN EXCLUSION” (the different one)

This one is different, and nobody warns people about it. An exclusion means your employer’s plan chose not to cover weight-management medication **at all**. There is no appeal for a benefit that was never purchased — and people spend months fighting this wall because it looks like the other two.

What to do instead

First, confirm it. Ask the plan directly: “Is this medication excluded from my plan, or denied for unmet criteria?” Those are different answers with different futures.

Then talk to HR, not the insurer. Exclusions are an employer purchasing decision, revisited every year at renewal. Ask whether next year’s plan will cover it. Employers track these requests — enough of them changes the decision, and you will not be the only one asking.

And talk to your prescriber about the whole picture. Coverage rules differ by diagnosis. Your prescriber knows your chart and what is honest to submit — that conversation belongs with them, not with a guide.

THE FIVE QUESTIONS TO ASK THE NUMBER ON THE LETTER

1. Is this an **excluded benefit**, or a denial for **unmet criteria**?
2. Exactly **which criteria** were not met?
3. What **documentation** would change the decision?
4. What is my **deadline** to appeal, and where does the appeal go?
5. If the appeal fails, how do I request an **external review**?

While you fight the paperwork: never stretch, split, or skip doses on your own to make a supply last. What happens with your medication during an appeal is a conversation with your prescriber — there are safe answers, and improvising isn’t one of them.

WHERE TO GO FROM HERE

A denial letter is a claim about paperwork, not a verdict on you. Find out which no you got — only one of the three is final.

If you're staring at the letter right now

Read it for the phrase: *step therapy*, *prior authorization*, or *exclusion*. Then make one call — the member line on the letter, with the five questions on the previous page. Ten minutes, and you will know more than most people ever learn about their own denial.

If your prescriber's office seems slow

Ask them directly: “Who in this office handles prior authorizations, and what do you need from me?” The office has a person for this. Find the person.

If coverage genuinely is not happening — or you don't want the fight

Telehealth is a legitimate route to a GLP-1 — **when the company is legitimate**. Before you pay anyone, check five things: a licensed clinician who actually reviews your health history, a required prescription, a named U.S. pharmacy you can verify, transparency about exactly which medication you're getting, and real follow-up care. A company that hides any of those is not a bargain at any price.

Not started yet, or not sure it's for you?

I'm a physician assistant, I've been on a GLP-1 myself, and the questions I get most are about side effects — not results.

Come find me: Instagram @diary_of_the_most_okayest_mom — I answer questions there, and the side-effect stuff nobody warns you about is most of what I post.

Educational only. This is not medical, insurance, or legal advice and it isn't a substitute for your own provider's judgment or your plan documents. Insurance rules vary by plan, employer, and state; deadlines and appeal rights are stated in your own denial letter, which is the authority on your case. Results are not guaranteed and individual results may vary.

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